How do you plan to integrate the TinyFish API?

Every fact a village health worker needs is already published by the government — **on portals with no API**. TinyFish web agents read those portals. Our existing safety rules decide what a clinician is allowed to see.

WHAT THE AGENTS READ

1 District

Live referral intelligence

eRaktKosh blood stock · state bed and ICU dashboards · PM-JAY empanelment

Ends referrals to a hospital that cannot treat this patient today

2 District

Medicine availability and price

Jan Aushadhi Kendra stock · NPPA ceiling prices

The doctor names a medicine the patient can actually obtain nearby

3 Pilot

Protocol freshness

MoHFW · IPHS · WHO guideline pages

Retrieval library stays current, with title, version and source URL

4 Pilot

Doctor credential check

NMC National Medical Register search

Telemedicine Guidelines 2020 require a registered practitioner

5 State

eSanjeevani hand-off

National teleconsultation portal — only with written authorisation

Complements the national platform instead of duplicating it

HOW IT SHIPS

Pilot — 1 PHC

Agents 3 and 4. Read-only, no patient data, no clinical risk.

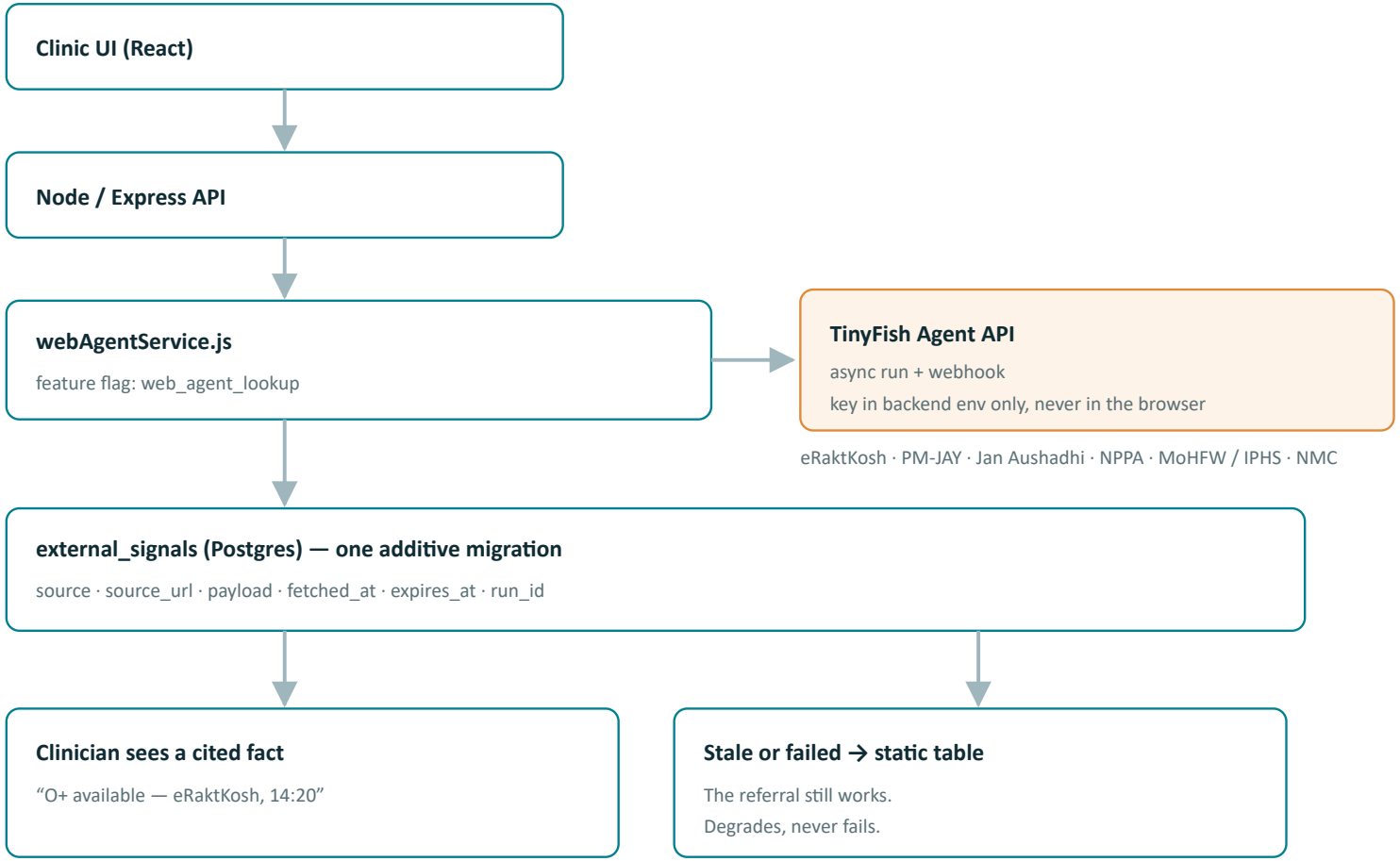
District — ~40 sub-centres

Agents 1 and 2, measured against the referral-completion baseline we already record.

State

Agent 5 with authorisation; webhooks feed the district outcome dashboard.

Architecture, and the three rules that keep it safe



THE THREE RULES

1. **No patient data leaves the platform**
   A query carries a district, a blood group, a facility. Never a name, an ABHA number, a complaint or a vital sign.

2. **A signal informs; it never decides**
   It cannot change the triage tier and cannot put a medicine on a prescription. Excluded from the learning loop and from FHIR exports.

3. **Cited and audited, always**
   The clinician sees the source and the time it was read; every fetch lands in the append-only audit log. A number with no source is never shown.

Cost: runs are scheduled per district, never per visit — spend scales with districts and refresh interval, not patient volume.